Chapter 10
Beauchamp and Childress: Principles of Biomedical Ethics

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10.1 Introduction

In 1979 Tom L. Beauchamp and James F. Childress set out what they considered the four basic principles of Biomedical Ethics. They are autonomy, nonmaleficence, beneficence, and justice—said to be the cornerstones of ethical decision-making. All four underscore human rights, particularly with respect to patient care. As we will see in this chapter, these four values also apply to solitary confinement and mental health concerns.

The first principle is autonomy. It is of fundamental significance in Western bioethics and the law. It is often held up as an ethical centerpiece, given the importance placed on individual decision-making.

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10.2 Principle #1: Autonomy

Beauchamp and Childress' first principle of Biomedical Ethics is autonomy, which they use to examine individuals' decision-making in health care and research. (2009, 99) Autonomy emphasizes self-rule (autos "self" and nomos "rule")— "free from both controlling interference by others and from certain limitations such as inadequate understanding that prevents meaningful choice." (Beauchamp and Childress 2009, 99)

In their view, most theories of autonomy see *liberty* (the independence from controlling influences) and *agency* (the capacity for intentional action) as the two essential conditions. (2009, 100) Obviously prisoners have limitations with regard to independence from controlling influences. To a great degree they lack the capacity for intentional action. Their autonomy is severely limited—particularly the case with those in solitary confinement. Their movement is constricted and opportunities for social engagement are negligible. In addition, they have few, if any, options for the use of their time. Inmates such as Albert Woodfox, remark on the boredom they suffer having nothing to do day by stultifying day. Some consider it a living death and become suicidally depressed and/or suffer hallucinations.

Just as it does for all of us, autonomy is crucial for our well-being. Being locked away from all human contact reduces chances of autonomy. Having more autonomous decision-making would improve the living conditions of those held in solitary.

The physical environment alone limits personal autonomy. Existing in a space the size of an eight-by-twelve-foot closet restricts physical movement to pacing back and forth and doing push-ups. Such an extreme living space and limited movement is hard to comprehend. It would certainly be unbearable.

Even worse are the social conditions. Correction: the *lack* of social conditions. Silence is the existential reality of solitary confinement. Those confined to solitary who yearn for a conversation often resort out of desperation to talking to themselves. That is why one inmate said he was "trapped in emptiness." So it is that policies and practices can work to compromise autonomous choice. Beauchamp and Childress observe that,

> The threat of exploitation is substantial in institutions where populations are admitted involuntarily, yet the fact that a person is coercively institutionalized or lives in a coercive institution does not automatically entail that each of that person's decisions is coerced. No reason exists why prisoners, for example, cannot validly consent to some research if researchers do not use coercive tactics in recruitment or make manipulative offers, such as unduly large payments for excessive risk taking. (2009, 135)

Bioethicist Jonathan D. Moreno discusses the use of prisoners in human subjects research in his book *Undue Risk: Secret State Experiments on Humans*. For example, prisoners were used in experiments on dioxin, malaria, hallucinatory drugs, and high-dose radiation, as well as other experiments undertaken by the U.S. government. Researchers decided in some cases that "only prisoner volunteers would satisfy the experimental requirements." (Moreno 2001, 30) Moreno points out that

"The inmates seem to have been thought suitable for some of the most dangerous and unpleasant tasks." (2001, 33)

Inmates were "thought suitable" because of their lower moral status. That the experiments were out of the public eye was likely another factor. Basically, the subjects were treated as expendable. "By the 1960s, at least half the states allowed prisoners to be used as medical guinea pigs," reports journalist Mike Stobbe for *NBC News*. (2011) Considering the value of personal autonomy, the essential conditions of liberty and agency should have been acknowledged. So, too, with issues involving informed consent.

A key ruling in the legal history of informed consent is *Schloendorff v. Society of New York Hospital* (1914) which established the right of self-determination. The New York Appeals Court ruled: "Every human being of adult years and sound mind has as a right to determine what shall be done with his own body." This decision put patient autonomy and self-determination on the map. It has continued to be a mainstay in court decisions involving informed consent.

Beauchamp and Childress point out the importance of autonomy in the work of both Mill and Kant, two of the most influential philosophers (see Chaps. 5 and 6) Specifically,

> Kant argued that respect for autonomy flows from the recognition that all persons have unconditional worth…To violate a person's autonomy is to treat that person merely as a means; that is, in accordance with others' goals without regard to that person's own goals… Mill's position requires both not interfering with and actively strengthening autonomous expression…. In their different ways, these two philosophers both support a principle of respect for autonomy. (2009, 103–104)

As Kant indicates, respect for autonomy is rooted in the knowledge that each individual has self-worth—as an end in themselves. It follows that prison staff, researchers, and medical personnel have a duty to honor the right of self-determination. That means prisoners have the right to refuse treatment and the right to withdraw from medical treatment or experimentation. These rights have been violated many times over. Jonathan Moreno cites numerous examples in his book *Undue Risk*. He points out the need to recognize ethical and legal boundaries with respect to personal autonomy and informed consent.

Furthermore, according to the IMAP/OSF Task Force, in 2004 policy at Guantanamo Bay allowed interrogators to use medical and psychological information about detainees to exploit their weaknesses in interrogation. Moreover, "It was reported the military interrogators were freely, assessing detaining medical records." (2013, xxii)

Individuals in solitary are at the mercy of others—from prison administrators to guards and medical personnel. Their autonomy and right of informed consent have not been given the recognition they deserve. All competent patients should have the right to refuse treatment. In detention facilities like Bagram, Abu Ghraib, and Guantanamo Bay this includes the right to refuse such degrading or humiliating practices as forced nudity, "rectal hydration," sleep deprivation, and force-feeding. Beatings were often common. "They used to tie us to this chair, our hands and feet, and then applied electric shocks. Sometimes they used it for beatings, too," detainee

Hajimumin Hamza says, recounting the torture he underwent during his captivity in Bagram prison between 2017 [and 2021], as Agnieszka Pikulicka Wilczewska reported for *Al Jazeera* (2021).

In her *New Yorker* article "The Experiment," Jane Mayer discusses the treatment of detainees at Guantanamo Bay. For example, detainee Murat Kurnaz contended that he had been "forcibly injected with unknown and debilitating drugs" (denied by U.S. authorities). His attorney, Baher Azmy, observed that, "The whole place appears to be one giant human experiment." (Mayer 2005)

The fundamental requirement of health professionals, Beauchamp and Childress assert, is to respect a person's autonomous choices, whatever they may be. Mayer notes that detainee lawyers had difficulty getting access to prisoners' medical records. (2005) Rob Kirsch, a representative for six detainees, contended that medical care is sometimes withheld or dispensed depending on a detainee's willingness to talk to interrogators (Mayer 2005).

"Respect for autonomy is not a mere *ideal* in health care; it is a professional *obligation.* Autonomous choice is a *right*—not a *duty*—of health professionals," state Beauchamp and Childress (2009, 107). Furthermore, "Prisoners, even terrorists, have a right to their medical records, according to federal laws, common laws, the American Medical Association, and court trials," as bioethicist Arthur Caplan points out (cited by Mayer 2005). With that in mind, let us turn to Beauchamp and Childress' second principle of Biomedical Ethics.

10.3 Principle #2: Nonmaleficence

The duty of nonmaleficence is in accordance with the Hippocratic Oath maxim "Do no harm." That sets a minimal standard of behavior: Whatever you do for your patients, at least do not harm them. The duty of beneficence (Principle #3) sets a higher standard by calling on medical personnel to maximize benefits for their patient and prioritize their well-being. Nonmaleficence and beneficence are companion principles, Beauchamp and Childress note. If possible, help others, do what you can to make an affirmative difference and address personal needs. If you cannot reach that standard, minimally do not harm them.

The message is clear. Medical personnel and prison staff pressured to enable abuse or mistreat inmates should adhere to the duties of beneficence and nonmaleficence. That means standing up to cruelty and mistreatment, intervening where possible, and reporting violations of human rights whenever they see them. By no means should prison wardens, guards, or healthcare professionals be complicit in abuse.

Beauchamp and Childress draw a distinction between harming and wronging. Wronging involves violating someone's rights as, for instance, if an insurance company improperly refuses to pay a patient's hospital bill, whereas harming a person may be a disease, a natural disaster, or bad luck (2009, 152). They point out some

specific rules that draw on the Principle of Nonmaleficence. Examples they cite are (2009, 153):

1. Do not kill.
2. Do not cause pain or suffering.
3. Do not incapacitate.
4. Do not cause offense.
5. Do no deprive others of the goods of life.

Solitary Watch (2023) reports on the harmful conditions individuals face—and must endure—in solitary confinement:

- The material conditions of solitary confinement units force individuals into chronic ennui and idleness.
- The cells are usually smaller than a parking space, with small, high windows or are windowless.
- Some cells have fluorescent lighting that never turns off, while others are in total darkness.
- In addition to being deprived of visual and auditory stimuli, victims of solitary confinement are denied opportunities for intellectual stimulation.
- Extracellular programming (such as work or education) is usually forbidden, as are reading and writing materials, radios, and art supplies.

In their discussion of nonmaleficience, Beauchamp and Childress consider major life-death dilemmas that physicians face caring for their patients. These include withholding and withdrawing treatment, extraordinary (or heroic) vs. ordinary treatment, and killing vs. letting die. One issue they look at is physician-assisted death. What are the moral boundaries in active euthanasia; for example, in assisting a patient in dying? Assuming the request is voluntary, and the patient is competent, would it be justified to accede to their wishes? Would that be a "death with dignity"? How much pain and suffering should count when considering physician involvement? Need an imminent death be in the prognosis?

When looking at treatment vs. non-treatment, Beauchamp and Childress advise, "The principal consideration should always be whether a treatment is beneficial or burdensome, not what its classification is." (2009, 159) They argue that "The distinction between optional and obligatory treatment, as determined by the balance of benefits and burdens to the patient, is the pertinent distinction." (2009,159)

10.4 Applications to Solitary Confinement

This matter has greater application to those in solitary confinement than we might expect. For individuals in prolonged isolation, such as those like Ted Kaczynski with a life sentence in solitary confinement, the extreme segregation might drive them to attempt suicide, as statistics have borne out. Kaczynski did commit suicide. The fact is that the rate of suicide is significantly greater for those in solitary than not.

This raises an important question facing medical caregivers. Could a quality-of-life argument support physician-assisted suicide for those in solitary confinement? Consider the argument:

The Argument for Assisted Suicide for Those in Solitary Confinement

- Prolonged solitary confinement (more than 15 days) has been recognized by the U.N. as torture.
- To be condemned to a lifetime of torture is morally unacceptable.
- Anyone facing a lifetime of torture has a liberty right of autonomy and nonmaleficence to opt out.
- Quality-of-life considerations support the right to assisted suicide.
- In recognizing that right, assisted suicide should be permitted.

Before we go further let us recall what Senator John McCain had to say: "It's an awful thing solitary," he said, "It crushes your spirit and weakens your resistance more effectively than any other form of mistreatment." (as cited by Fetig 2011) It fails the quality-of-life test, particularly because of the lack of human contact.

McCain (2008) also said, "As far as this business of solitary confinement goes—the most important thing for survival is communication with someone, even if it's only a wave or a wink, a tap on the wall, or to have a guy put his thumb up. It makes all the difference." (2008)

To consider assisted suicide a legitimate right, we should factor in two issues. First, that this option differs from the death penalty, which is is a *terminal* punishment meant to fit the crime. It is *not* an act of autonomy—of preferring to die rather than enduring a life of torture. In addition, the death penalty is a decision made by others and not an autonomous choice on the part of the individual considering suicide.

Secondly, this is a quality-of-life decision. Those confronting a lifetime of the torture of solitary confinement should have the right to exercise their autonomy. And it is not just an issue of low social worth—it is also the lack of a meaningful existence. And, so, assisted suicide may be thought desirable for those in solitary, cut off from human contact. However, that conclusion calls for closer consideration. In Beauchamp and Childress' view, we can avoid opening the door to active, involuntary euthanasia by stating the criteria for a quality of life with enough precision to avoid that danger (2009, 170).

One way is not to confuse quality-of-life for the patient with the value of the patient's life for *others*, as Beauchamp and Childress emphasize (2009, 170). Though the general public may be indifferent to the subjects' quality-of-life concerns, the families of those in solitary are not. They have a role to play. "The President's Commission for the Study of Ethical Problems in Medicine and Biomedical and Behavioral Research recognize a broader conception of best interests that includes the welfare of the family." (Beauchamp and Childress 2009, 170–171)

The Commission points out that the *impact* of a decision may be considered in determining someone's best interests (Beauchamp and Childress 2009, 171). If the

impact on the family weighs in, neither the loved one facing a life of torture in solitary nor undergoing an assisted death should factor into the equation. What then?

At this point we must ask: "Is there an alternative that bears consideration—a third possibility for those in solitary? The answer is "Yes." Per the recognition that prolonged solitary is a form of torture, surely there is a better course of action than choosing to die. Assisted suicide does not seem to qualify as "death with dignity" in this scenario. Is there any dignity in wanting to end your life because it is intolerable—physically, mentally, and psychologically?

10.5 The Argument for an Alternative to Assisted Suicide

The quality of life of those in solitary is unacceptably brutal due to the physical conditions of the limited space, the mental harms, and the lack of human contact. There are other ways to punish or confine someone without turning to the extreme of solitary confinement. After looking at Beauchamp and Childress' remaining two principles, we will return to this issue.

10.6 Principle #3 Beneficence

Beauchamp and Childress's third principle is beneficence. Whenever possible, they suggest, we should strive to do good. That duty is a call for moral excellence and for doctors and other medical personnel to do what they can to help their patients. Similarly, prison wardens and guards should show beneficence in their treatment and interaction with their inmates. At no point should they enable abuse or deceit—their own integrity is at stake.

Principles of Beneficence demand more than the Principle of Nonmaleficence "because agents must take positive steps to help others, not merely refrain from harmful acts," argue Beauchamp and Childress (2009, 197). They focus on two principles of beneficence: positive beneficence and utility. *Positive beneficence* requires providing benefits to others, whereas *utility* requires balancing benefits, risks, and costs for the best overall results (2009. 197). "In ordinary English, the term *beneficence* connotes acts of mercy, kindness, and charity… *Benevolence* refers to the *character trait* or *virtue* of being disposed to act for the benefit of others." (Beauchamp and Childress 2009, 197) That is an important quality to strive for.

Beauchamp and Childress observe that we are not morally required to perform *all* possible acts that would benefit others, as much beneficent conduct constitutes an *ideal*, rather than an obligation (2009, 199). Nevertheless, some of the moral rules of obligation are:

1. Protect and defend the rights of others.
2. Prevent harm from occurring to others.

3. Remove conditions that will cause harm to others.
4. Help persons with disabilities.
5. Rescue persons in danger.

Rules 2 and 3 are particularly applicable to solitary confinement, given the degree and duration to which it causes harm. Beauchamp and Childress' Principle of Beneficence calls for a need to address harm. As they suggest, rules of beneficence present positive requirements of action. "We are morally prohibited from causing harm to anyone." (2009, 199) This includes self-harm as well.

Inmates are not exempt from this prescription. That said, we are obligated to act nonmaleficently toward everyone, but we are not required to act beneficently toward *all* people. "Nonetheless," they point out, "we are obligated to follow impartially *some* rules of beneficence, such as those requiring efforts to rescue strangers when the rescue efforts pose little risk." (2009, 199) It would seem that acting beneficently toward those in solitary similarly poses little risk. Therefore, we should consider it morally advisable. We should also take steps to address self-harm as well.

According to Solitary Watch (2023), one study showed that people in solitary are seven times more likely to commit self-harm and six times more likely to commit fatal self-harm than those held in general prison populations. People held in solitary for any amount of time while serving a prison sentence are more likely to die of all causes in the first year after release, especially of suicide and homicide (Solitary Watch 2023). Clearly these are alarming statistics.

10.7 Beneficence and Assisted Suicide

Beauchamp and Childress cite a case that raises interesting issues. It centers on the first person—Barney Clark—to receive an artificial heart. Evidently Clark was given a key that allowed him to turn off the compressor if he wanted to die. He thereby had the means and the recognized autonomy to end his life if he felt "It isn't worth it anymore." (2009, 217)

Beauchamp and Childress ask if Clark's use of the key would have been an act of suicide. If he had refused the artificial heart in the first place few would have labeled his act a suicide. On the other hand, if he shot himself after getting the artificial heart, that act would be thought a suicide (2009, 217).

They concluded that, if Clark used the key and turned off the heart, the controversy would be whether it was an act of suicide, forgoing life-sustaining treatment, withdrawing from an experiment, or all of the above (2009, 217). They contend that, "If suicide is a protected moral right, then the state, health professionals, and others have no legitimate grounds for intervention in autonomous suicide attempts." (2009, 217) Autonomy rules.

Does it then follow that persons in solitary have a moral right to an *assisted* suicide?

If suicide is a legitimate choice, then there are few, if any, reasons not to permit an assisted suicide. However, it could be that no health care professional is willing to take part in an assisted suicide due to their own values and beliefs. What then?

Rather than seeing their assistance as a duty, there is a better choice; namely addressing the underlying despair that leads to suicidal thoughts in the first place. Surely there is a palliative care alternative to either continuing the solitary confinement or committing suicide. It bears concern for the well-being of the inmates/patients and the medical personnel as well. Health sciences professor Amber Comer points out the position of the AMA:

> The American Medical Association (AMA) Code of Medical Ethics does not condone physician participation, calling the practice "fundamentally incompatible with the physician's role as healer." As an alternative to PAD [physician-aid-in-dying], the AMA advocates aggressive multidisciplinary interventions including emotional support and adequate pain control (2019).

At this fork in the road, we would benefit from putting Beauchamp and Childress' fourth principle in the spotlight. Let's go there next.

10.8 Principle #4: Justice

Beauchamp and Childress's fourth principle is justice. This is in line with Aristotle, who considers justice to be the overarching moral duty—one with vast significance. What follows from this? Well, justice entails fairness and consideration of all concerned. Philosopher Wayne P. Pomerleau offers this interpretation: "Aristotle says justice consists in what is lawful and fair, with fairness involving equitable distributions and the correction of what is inequitable." (2013) There are three aspects to this definition; namely what is fair, what is lawful, and what is equitable.

Beauchamp and Childress point out that "The terms *fairness, desert* (what is deserved), and *entitlement* have been used by various philosophers in attempts to explicate *justice*." (2009, 241) As with Aristotle, they see justice as fair, equitable, and appropriate treatment due to people. In contrast, "An injustice involves a wrongful act or omission that denies people resources or protections to which they have a right." (Beauchamp and Childress 2009, 241)

Whenever possible, doctors, prison wardens, guards, and other staff should intervene to stop abuse and other injustices and do all they can to change the status quo. At times inmates have been deprived of solid food from days to months and many have been subjected to sexual abuse, sleep deprivation, stress positions, and more mistreatment.

Distributive justice includes "policies that allot diverse benefits and burdens such as property, resources, taxation, privileges, and opportunities." It refers to the distribution of all rights and responsibilities in society (Beauchamp and Childress 2009, 241). A "compelling example" that they cite involves human subjects in research—an issue we considered earlier in this chapter.

Up into the 1990's, note Beauchamp and Childress, concerns centered on risks and burdens of nontherapeutic research. With the advent of HIV/AIDS, therapeutic uses of research broadened the justice considerations. As a result, access to research became as important as protection from exploitation (2009, 241). "*Freedom to"* and "*freedom from"* were both seen as aspects of justice. With that expansion, the importance of autonomy, nonmaleficence, and beneficence were underscored.

Beauchamp and Childress point out the following principles that tied to distributive justice (2009, 243):

1. To each person an equal share
2. To each person according to need
3. To each person according to effort
4. To each person according to contribution
5. To each person according to merit
6. To each person according to free-market exchanges

Principles 2 and 5 are particularly applicable to those in solitary confinement. In light of the torturous conditions, those subjected to the practice need help. Beneficence is definitely in order. The subjects very humanity, as Kant would recognize, merits more attention with respect to harms than has been paid. That can change.

At this point Beauchamp and Childress turn to John Rawls. Rawls called for a framework of justice universalizing human rights and a system of liberty accessible for all. (see Chap. 8) As much as possible eliminate bias and prejudice in order to help those who are most disadvantaged. This understanding of justice has repercussions in terms of duties to uphold. That should include standing up against long-term segregation and isolation, which the U.N. recognizes as a form of torture. Rawls would recommend a stronger role for justice than has been the case in prisons for decades. It's time for an overhaul.

Rawls has the most celebrated egalitarian theory of justice, asserts Beauchamp and Childress. He argues that people should receive "an equal distribution of certain goods such as health care." (2009, 247). That is rooted in Rawls' first justice principle—to universalize human rights. Inequalities (as with income) are allowed only if they benefit everyone and are based on fair equality of opportunity (2009, 247).

Disparities like racism and sexism fall under the second principle—the fair-opportunity rule. And they are a problem. Beauchamp and Childress point out note that "Rawls considers social institutions—including prisons—to be just if and only if they conform to these two basic principles (2009, 247). They observe:

> Many studies in the United States indicate that Blacks and women have poorer access to various forms of health care in comparison to white males. For example, gender and racial inequities in employment have an impact on job-based health insurance; and the race and gender of physicians often play a role in the quality of patient-physician interaction (2009, 250).

There is no doubt that both racism and solitary confinement violate Rawls' two principles. Rawls would be aghast at what he would see in the conditions and treatment of those in solitary confinement.

10.9 Justice and Racism

The National Commission on Correctional Health Care cast a critical eye at the disparities and inequalities affecting those in prison and definitely those in solitary confinement. Take racism, for example:

> In the United States, Black people are disproportionately harmed by the criminal legal system, from from violent encounters with police to facing harsher bail setting and sentence length than white people. There is a distinct through-line in American history, beginning with slavery and continuing through vagrancy laws, convict leasing, sharecropping, and the "War on Crime" and "War on Drugs," that illuminates systemic factors leading to the disproportionate incarceration of Black people relative to white people.
>
> The U.S. population is comprised of about 13% Black people, well about 35% of jail residents are Black. … Black people have a lower life, expectancy relative to white people and fare worse than white people on a number of health indicators, including receipt of mental health care (The National Commission on Correctional Health Care 2024).

That's not all. Many prisons treat inmates like slave labor. Look, for instance, at the Draper Correctional Facility in Elmore, Alabama. Prisoners take part in a "work-release" labor program for private businesses. This work is not optional, as they risk disciplinary action if they refuse, reports Talmon Joseph Smith for *The New York Times* (2024). "Under Alabama prison rules, there are thin lines between work incentives, forced labor and "involuntary servitude." (Smith, 2024)

Inmate Carlos Anderson says, "You have no choice." "If you don't," he adds, "they're going to send you back to the camp and you get rolled up with a disciplinary charge… which prolongs incarceration or being made to work without pay at prison facilities." (Smith, 2024)

> Critics say the system perpetuates the racist legacy of "convict leasing," which lasted in Alabama from 1875 until 1928. It forced state prisoners to do uncompensated work for private companies, which then kicked back lease payment fees to the state. Prison labor, however, remains common, from California to Texas to New York,…
>
> Across the country, state and federal prisoners earn 13 to 52 cents per hour on average, ….Unpaid labor for inmates is still allowed in Alabama and a few other states. In Alabama, unpaid internal prison work can be assigned to those who have not qualified for or been assigned to work-release…. Some inmates work for state and local entities, at tasks like road maintenance, for about $2 a day—the rate Alabama set for prison laborers in 1927. (Smith, 2024)

That's not all. California voters rejected a measure in the 2024 election to ban forced prison labor. Journalist Sophie Austin notes that the constitution already prohibits so-called involuntary servitude, but an exception allows it to be used as a punishment for crime (2024). In contrast, Nevada voters passed the proposal.

10.10 Conclusion

The Commission on Human Rights of the United Nations asserts that "The right of everyone to the enjoyment of the highest attainable standard of physical and mental health is a human right…for millions of people throughout the world, the full realization of th[is] right…still remains a distant goal." (as noted by Beauchamp and Childress 2009, 264)

Philosophers Madison Powers and Ruth Faden put forth a basic premise about justice; namely, "Social justice is concerned with human well-being—health, personal security, reasoning, respect, attachment, and self-determination…. The justice of societies and of the global order can be judged by how well they implement these dimensions." (Beauchamp and Childress 2009, 265)

Beauchamp and Childress consider the job of justice—their fourth principle—to be closely associated with beneficence (2009, 265). We might note the degree to which justice is also tied to autonomy and nonmaleficence. The four basic values that Beauchamp and Childress highlight are not just basic—and vital—for Biomedical Ethics. They also offer guidance for the treatment—and harms—of solitary confinement.

We need to keep the duties of autonomy and nonmaleficence before us, hold tight to the duty of beneficence, and render help. That means standing up to unjust practices—intervening where possible, and reporting violations of human rights whenever we see them. By no means be complicit in abuse. There are lines that should not be crossed.

We should keep in mind what Jack Morris, a survivor of solitary confinement, has to say when we consider Beauchamp and Childress' four values and the importance of working for change.

> There are few forms of torture worse for the human soul than isolation. Solitary confinement is used to break people, and many who are exposed to it suffer permanent harms. It has been found to be damaging to the body, and the mind, and ultimately results in broader societal harms that have been documented (Morris 2024).

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